جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : وفاء عبد الحميد زكي الجارحي
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** 0
رقم البطاقة : 27810010109942 **رقم التليفون :** 01141949381 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 01-10-1978 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: وفاء عبد الحميد زكي الجارحي
National ID: 27810010109942

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

MDT FORM 30/12/2025 G

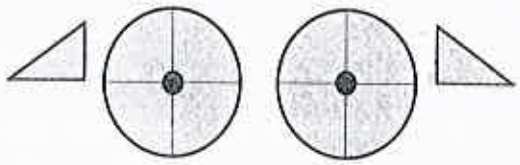
Name	وفاء عبدالحميد ذكي	MRN	1908/20
		ID	29811010101425
Age	47	Phone	01141949381
Menopausal status	pre	Marital status	married
NRS SCORE		Family history	+ve family
BMI		REASON OF MDT	Re Mdt
CO morbidity	medically Free		

Clinical Data

BX: IDC II

ER:	PR:	HER 2:	Ki67:
8/8	5/8	1+	5%

R L



TNM

Sonomammography: lt breast irregular mass seen at 10'oclock 21x16mm with another area of parenchymal distortion 12->1 o'clock.

Mets Workup: CESM: non mass area enhancement extend from posterior third of the breast to the nipple with overall coverage area: 11x9x6.5 cm.

CT CAP: free

Recommendations: for possible upfront lt SM + SLNB

MDT OUTCOME & PLAN

lt SM + SLNB for possible reconstruction at surgery clinic

~~After 4 cycles of~~ start NAC (AC x3 → Taxol x3) + Re-evaluate the SSM + AND

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

24/12/25

AC

Name: وفاء عبد الحميد زكي الجارحي **MR. :** 4531	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient was diagnosed with left-sided breast cancer and received neoadjuvant chemotherapy consisting of doxorubicin and cyclophosphamide for three cycles (AC × 3) followed by docetaxel for three cycles (TXT × 3) as the first anthracycline-based regimen, with an echocardiogram showing an ejection fraction of 66 %.

Pathology confirmed a left-breast invasive ductal carcinoma, clinical stage T2 N0 M0, grade 2, estrogen-receptor score 8/8, progesterone-receptor score 8/8, HER2-negative, Ki-67 5 %. Contrast-enhanced spectral mammography demonstrated both a mass and non-mass enhancement in the upper outer quadrant and upper inner quadrant. The patient experienced grade 2 vomiting during the first cycle of AC.

On the day of presentation the patient received cycle 1 of docetaxel at a dose of 75 mg/m², with supportive care and methabio-gen tablets pending laboratory results.

The patient returned for administration of cycle 1 docetaxel 75 mg/m², again with supportive care and continuation of methabio-gen tablets.

At the most recent visit the patient's weight was 93 kg and height 163 cm; physical examination revealed no palpable breast masses. The patient received cycle 2 of taxotere (docetaxel) at a dose of 150 mg. The patient has completed 2 cycles of taxotere.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL ACADEMY

Patient Name: Wafaa Abdel Hameed Zaky Al Garhy	Date : 24/09/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 7916	Pat. ID: 121703

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- **Breast density:** Scattered areas of glandular tissue....Matching ACR b.
- *Focal asymmetry and distortion with obscured margin mass lesion is seen in the left UOQ..*
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the right breast,
- Normal skin and nipple on both sides.

By complementary breast ultrasound:

Left breast:

- Homogenous glandular echo-texture.
- *Corresponding to the mammographic density seen in the left upper outer quadrant, there is an irregular shaped mass is seen at 1 o'clock zone 2 measuring (21 X 16 mm) with another area of parenchymal distortion is seen at 12-1 o'clock position zone 1& 2 .*
- *Normal left skin.*
- Normal left skin and nipple.
- Non specific axillary LNs.

Right breast:

- Homogenous glandular echo-texture.
- No detected suspicious solid or cystic masses.Normal right skin and nipple.
- No detected significant right axillary LNs.

CONCLUSION:

- *Left breast: Left UOQ suspicious mass with another area of distortion ?multifocal affection for post contrast study .... BIRADS 4b Category.*
- *Right breast: Early fibrocystic changes .....BI-RADS 2 Category.*

Much Obliged

DR.MONA ZAKY.MD

professor-Breast Imaging-Mansoura University

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	: الأستاذة / رقاء عبدالحميد زكي		
Patient No.	: 27810010109942	Request No.	: 13004273
Path No.	: 3041/25	Request Date	: 06/10/2025
Sex	: Female	Verification Date	: 13/10/2025
Age	: 47 Y	Financial Category	: جوستاف روسي - مباشر
Referreing Doc	:	Department	: Out Patient
		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast suspicious mass, BIRADs 4b.
Nature of Specimen: Tru-cut biopsy, collected on 05/10/2025.

Gross Examination:
- Three grey white tissue cores, the longest was 1.5 cm. Totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).
- DCIS: 20% Intermediate grade ductal carcinoma Insitu (Cribriform & clinging patterns).
- AngioInvasion: Present.
- Perineural invasion: Present.

- IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 2% of tumor cells (score 5/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).
- KI67%: 5%.

Diagnosis:
Left breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2, with 20% intermediat grade ductal carcinoma insitu (Clinging & cribriform patterns).
IPT:

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة .. القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE / OUSSY
INTERNATIONAL

Patient Name	: الأستاذه / رلاء عبدالحميد زكي	Request No.	: 13004273
Patient No.	: 27810010109942	Request Date	: 06/10/2025
Path No.	: 3041/25	Verification Date	: 13/10/2025
Sex	: Female	Financial Category	: جوستاف روسي - مباشر
Age	: 47 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

- ER: Positive (score 8/8).
- PR: Positive (score 5/8).
- Her2: Low (score 1+).
- Ki67%: 5%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL
ALEXANDRIA

Patient Name: Wafaa Abdel Hameed Zaky Al Garhy	Date : 21/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 47 Year(s)
Medical ID: 2952/2025	Pat. ID: 121703

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

Known case of CA breast.

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bone lesions.

Opinion:

❖ Known case of CA breast showing no CT evidence of metastatic disease.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

09/27/2026

MDT FORM

Name:	Wafaa Abdul Hamid Zaky Al-Jarhi	**MRN:**	4531
Age:	48 Y	**ID:**	27810010109942
Menopausal Sta	pre	**Phone:***	01146041514
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:	m.free	**Reason Of MDT:**	post op
Surgical History:*	nad	**ECOG Performance Status**	○ 0 ○ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** -VE **KI67:** 15%

History:
sp
lr breast post nact ltcbs& slnb
skin free
surgical margins >> free
slnb 0/6
intra mammary ln 0/1
pathological partial response therap ypT1c, ypN0

Sonomammography:

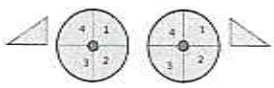
Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan
FOR ADJ FEMARA 5 YEARS
PORT

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د حنان عزت	د احمد نجيب		

MDT Form
[[DOCTYPE:MDT_FORM]]